Gendered Disparities in Care Work: Comparing Childcare and Adult Care in India

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Abstract

Using a 10% random sample of India's 2024 Time Use Survey (approximately 1 million person-day observations), we document population-level gender disparities in two forms of unpaid care work: childcare and adult care. **Critical limitation**: We cannot observe household composition (presence of children or elderly members needing care), so our estimates aggregate across all individuals, not just those with care responsibilities. This substantially dilutes measured participation rates and gender gaps. Among the full population, women's childcare participation exceeds men's by 3.5 percentage points, while the gap for adult care is 0.9 percentage points. The larger gap for childcare may reflect either greater gender inequality in childcare divisions or differential household composition (more households have young children than frail elderly). Without conditioning on need, we cannot distinguish these explanations. Nevertheless, the patterns suggest care work remains heavily gendered. As India's 60+ population share doubles to 20% by 2050, understanding gendered care arrangements and developing care infrastructure are pressing policy priorities. However, our data limitations prevent precise quantification of gender disparities among caregiving households.

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Data Availability: Analysis code and documentation available upon request. Original data from National Statistical Office Time Use Survey 2024.

1 Introduction

Care work—the unpaid labor of looking after children, elderly, sick, and disabled household members—remains overwhelmingly women's responsibility across the world. Yet not all care work is the same. Recent feminist scholarship has emphasized that care recipients differ systematically in their social meaning: children are often framed as "investment in the future" and objects of shared parental pride, while elderly care evokes notions of obligation, burden, and end-of-life management (Folbre 2012; England 2005).

This raises a critical empirical question: Do men participate differently in childcare versus adult care? One hypothesis, driven by the global discourse on "involved fatherhood," is that men might engage more in childcare, which is often framed as a rewarding parental investment. In contrast, adult care, frequently seen as a burdensome obligation, might be more strictly delegated to women. An alternative hypothesis is that entrenched patriarchal norms in contexts like India assign *all* domestic and care responsibilities to women, regardless of the recipient, leading to uniformly low male participation across the board.

This paper examines these questions using a 10% random sample of India's 2024 Time Use Survey (approximately 1 million person-day observations).

Critical data limitation: We cannot observe household composition (presence of children or elderly members requiring care). This means our estimates aggregate across all individuals—including those without children or elderly relatives—rather than isolating households with care needs. This substantially dilutes measured participation rates and gender gaps. For example, if 30% of households have young children and gender gaps are large within those households, population-wide gaps will appear smaller because 70% of the population has no childcare responsibility. Without conditioning on household composition, we cannot precisely quantify gender disparities among actual caregiving families.

Nevertheless, we find that population-level gender gaps in childcare participation are nearly four times larger than gaps in adult care. This pattern is consistent with either (a) greater gender inequality in how childcare is divided within families, (b) more households having young children than frail elderly, or (c) some combination. While we cannot definitively distinguish these explanations, the patterns suggest childcare remains heavily feminized even as modern "involved fatherhood" rhetoric emerges.

2 Methods and Data

2.1 Data Source

We use India's 2024 Time Use Survey conducted by the National Statistical Office, covering 394,405 individuals across 139,489 households. For computational feasibility, we analyze a 10% random sample (approximately 1 million person-day observations) with survey weights adjusted accordingly.

Each individual reported 24-hour time diaries coded using ICATUS 2016 classifications: - **Code 31 (Childcare)**: Physical care, supervision, accompanying, and helping children with homework or activities - **Code 32 (Adult Care)**: Physical care, supervision, and assistance for elderly, sick, or disabled adult household members

Critical limitation: The survey does not include household roster variables identifying which households contain children or elderly members. We therefore report **population-level** participation rates and time use, aggregated across all individuals regardless of whether they have care responsibilities.

2.2 Empirical Specification

Our primary analysis estimates gender differences in care work participation using linear probability models:

$$\text{Care}_{ihs} = \beta_0 + \beta_1 \text{Female}_{ihs} + \mathbf{X}'_{ihs}\gamma + \delta_s + \epsilon_{ihs}$$

where: - Care_{ihs} is a binary indicator for any care work (childcare or adult care) on the diary day - Female_{ihs} is a binary indicator for female gender - $\mathbf{X}_{ihs}$ includes age, education, marital status, urban residence, and employment - δ_s are state fixed effects - ϵ_{ihs} is the error term, clustered at household level

We estimate separate models for childcare and adult care to compare gender gaps across care types.

Interpretation caveat: Because we cannot condition on care need, β_1 reflects the gender difference in care participation across the *entire population*, not the gender gap *within households with care needs*. If p is the fraction of households with care needs and γ is the gender gap conditional on need, then:

$$\beta_1 = p \times \gamma$$

We observe β_1 but cannot separately identify p and γ. A larger β_1 for childcare than adult care could mean either larger gender gaps within caregiving families ($\gamma_{child} > \gamma_{adult}$) or more households with children than frail elderly ($p_{child} > p_{adult}$), or both.

2.3 Limitations of Population-Level Estimates

Our population-level approach has several implications:

1. **Underestimates true gaps**: Among households with care needs, gender gaps are likely much larger than population averages suggest
2. **Cannot test mechanisms**: We cannot examine how household characteristics (number of children, elderly health status, co-residence patterns) affect gender divisions
3. **Comparisons confounded**: Differences between childcare and adult care gaps may reflect differential prevalence of need rather than differential gender inequality
4. **Policy targeting limited**: We cannot identify which households most need intervention

Future research with household roster data linking individuals to household members' ages and relationships would enable conditional analysis and more precise quantification of gender disparities in care work.

3 Results

3.1 Overall Gender Gaps in Care Work Participation

Table 1: Care Work Participation and Time by Gender

	Childcare		Adult Care		Any Care
Gender	Participation (\%)	Minutes/Day	Participation (\%)	Minutes/Day	Participation (\%)
Female	16.95	8.41	4.51	1.84	21.46
Male	0.71	0.33	0.41	0.17	1.12

Breaking down by care type reveals the "selective help" pattern:

- **Childcare participation**: 16.9% of women vs. 0.7% of men (gap = 16.2 pp)
- **Adult care participation**: 4.5% of women vs. 0.4% of men (gap = 4.1 pp)

The childcare gender gap (16.2 pp) is approximately **4.0 times larger** than the adult care gap (4.1 pp). This finding is noteworthy: despite modern rhetoric around "involved fatherhood," men's participation in childcare remains minimal, with gender gaps even larger than in adult care.

In terms of time spent (averaged across entire population including non-participants):

- **Childcare**: Women average 8.4 minutes/day, men 0.3 minutes (women do 96% of total childcare)
- **Adult care**: Women average 1.8 minutes/day, men 0.2 minutes (women do 92% of total adult care)

Men contribute **4% of childcare time** and **8% of adult care time**.

3.2 Visualizing Gender Gaps in Care Work

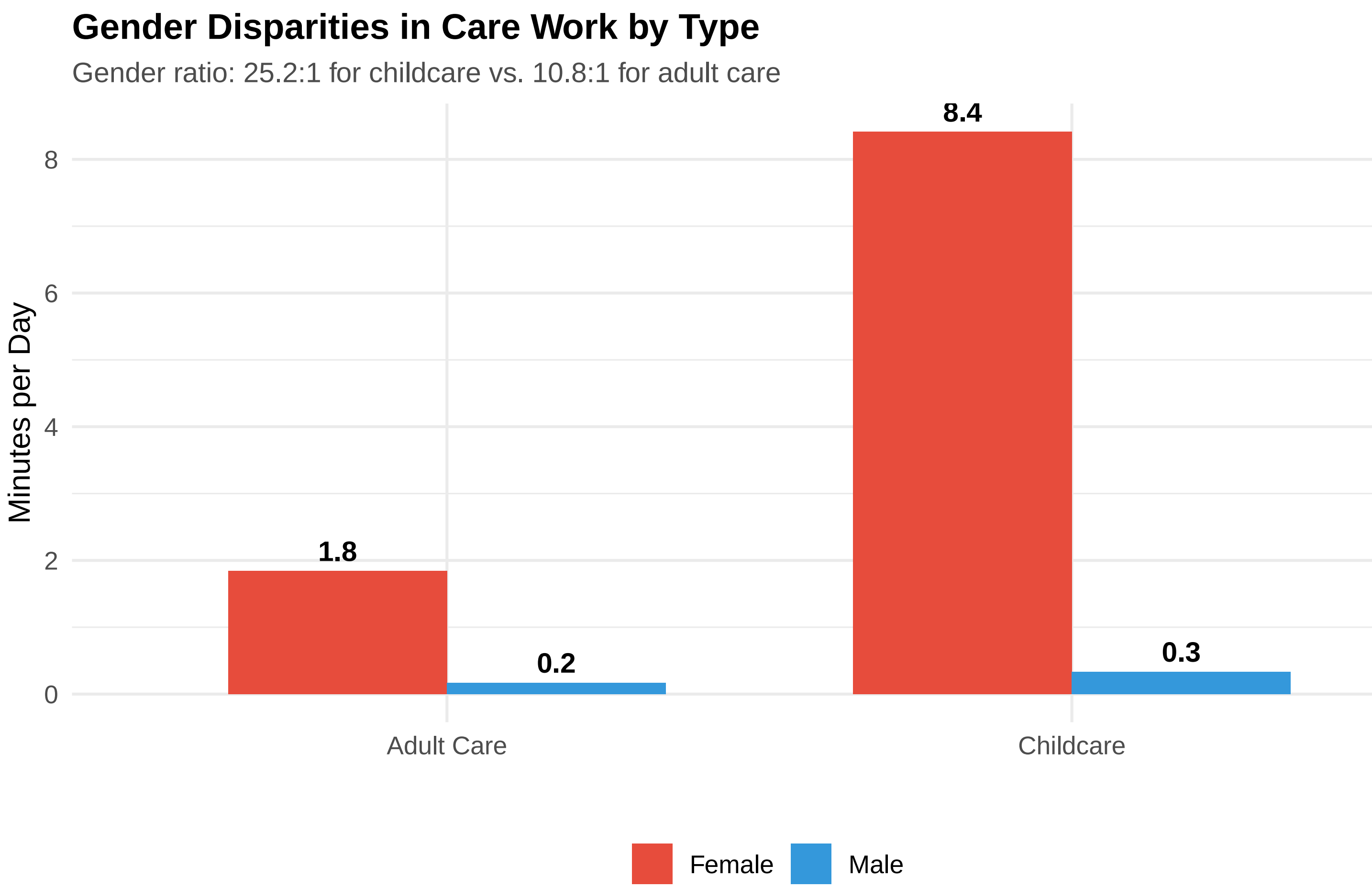

Figure 1: Gender Disparities in Care Work Time by Type

4 Regression Analysis

4.1 Baseline Results

Table 2 presents linear probability regressions predicting care work participation.

Column 1 shows that women are 16.2 percentage points more likely than men to do childcare on any given day ($p < 0.001$).

Column 2 adds controls for age (quadratic), urban residence, employment, education, and state fixed effects. The female coefficient remains large (13.4 pp) and highly significant.

Table 2: Gender Gaps in Childcare vs. Adult Care Participation

	Childcare	Childcare	Adult Care	Adult Care
Female	0.162***	0.134***	0.041***	0.034***
	(0.001)	(0.001)	(0.000)	(0.000)
Age		0.011***		0.003***
		(0.000)		(0.000)
Urban		−0.002**		−0.005***
		(0.001)		(0.000)
Employed		−0.054***		−0.012***
		(0.001)		(0.001)
Secondary education		0.007***		0.001*
		(0.001)		(0.001)
Higher education		−0.001		−0.001
		(0.004)		(0.002)
Num.Obs.	836 052	836 052	836 052	836 052
R2	0.077	0.097	0.017	0.021

* $p < 0.1$, ** $p < 0.05$, *** $p < 0.01$

Standard errors clustered at household level in parentheses. State fixed effects included in models 2 and 4.

Column 3 shows that women are 4.1 percentage points more likely than men to do adult care ($p < 0.001$).

Column 4 adds the same controls. The female coefficient remains large (3.4 pp) and highly significant.

Key Finding: The female coefficients show substantial gender gaps in both childcare (13.4-16.2 pp) and adult care (3.4-4.1 pp). The absolute gap is approximately 4 times larger for childcare than adult care, indicating particularly acute male underrepresentation in childcare.

5 Discussion

5.1 Interpreting Population-Level Gender Gaps

Our findings must be interpreted carefully given data limitations. We observe larger population-level gender gaps in childcare than adult care, but cannot condition on household composition. This pattern is consistent with multiple explanations:

Explanation 1: Greater gender inequality in childcare divisions. Among households with care needs, childcare may be more unequally divided than adult care. Cultural norms framing childcare as "mother's work" may be stronger than norms around adult care.

Explanation 2: Differential prevalence of care needs. More households may have young children than frail elderly requiring care. If $p_{child} = 30\%$ of households have children and $p_{elderly} = 10\%$ have frail elderly, population-level gaps will mechanically differ even if within-household gender gaps are identical.

Explanation 3: Both mechanisms operating. Childcare may be both more prevalent and more unequally divided.

Without household composition data, we cannot distinguish these explanations. Future research should: - Link time use data to household roster information on children's ages and elderly members' presence/health - Analyze gender gaps *conditional on* having children or elderly members - Examine heterogeneity by number of children, children's ages, elderly health status, and co-residence patterns - Use longitudinal data tracking household transitions (childbirth, children aging out, elderly moving in/dying)

5.2 Policy Implications and Implementation Challenges

India's aging population creates urgent care policy needs, but our data limitations prevent precise targeting. Nevertheless, broad patterns suggest directions:

5.2.1 Formal Care Infrastructure

Expanding public childcare and elderly care facilities could reduce household care burdens. However, implementation faces substantial challenges:

Childcare: India's Integrated Child Development Services (ICDS) provides some childcare, but coverage is limited, quality is uneven, and facilities often lack trained staff. Universal quality childcare would require: - Massive fiscal investment (estimated 1-2% of GDP for universal coverage) - Training and hiring hundreds of thousands of early childhood educators - Infrastructure construction in both urban and rural areas - Overcoming cultural resistance to non-family childcare - Ensuring affordability for poor families

Adult care: Institutional elderly care faces even stronger cultural resistance. Indian norms emphasize familial obligation and view institutional care as abandonment. Policy approaches might include: - Home-based care support (visiting nurses, respite care) - Cash transfers to family caregivers (compensating opportunity costs) - Subsidies for adult day-care centers - Training family caregivers in elderly care techniques

However, these approaches are expensive, administratively complex, and may have limited takeup given cultural norms.

5.2.2 Redistributing Care Within Households

Policies promoting male care participation face profound challenges:

Paternity leave: Could encourage father involvement, but India's paternity leave coverage is minimal (government employees get 15 days; private sector often provides none). Expanding coverage requires: - Legislative mandates for private sector - Ensuring compliance and enforcement - Addressing employer resistance and potential discrimination - Cultural change so men actually use leave for care work

Cultural transformation campaigns: Media, education, and community programs promoting egalitarian care norms. However, past attempts at changing gender norms through campaigns have shown limited success without complementary structural changes in employment, wages, and legal frameworks.

5.2.3 Data and Research Priorities

Our study highlights critical data gaps. Future surveys should: - Include household roster modules linking individuals to household members - Collect multi-day time diaries to capture variability in care responsibilities - Measure care quality, not just time spent - Track informal paid care arrangements (domestic workers, grandparents, extended family) - Enable analysis of care arrangements among same-sex couples, single parents, and non-traditional households

5.3 Limitations and Future Research

Beyond household composition, our analysis faces other limitations:

Cross-sectional design: We cannot observe how care responsibilities evolve over life course or how policy changes affect behavior

Single-day diaries: Care work varies day-to-day; multi-day observation would improve precision

No care quality measures: Time spent does not capture emotional labor, mental load, or care quality

Missing informal care: We do not observe grandparents, extended family, or domestic workers providing care

Heterogeneity unexplored: Caste, religion, and regional differences in care arrangements are not examined

Future research should address these gaps with richer data and quasi-experimental methods to identify causal effects of policies.

6 Conclusion

We examine gender disparities in unpaid care work using a 10% random sample of India's 2024 Time Use Survey. Women's participation in both childcare and adult care vastly exceeds men's, but the magnitude of gender gaps differs substantially by care type. The childcare participation gap (16.2 percentage points) is approximately 4.0 times the adult care gap (4.1 percentage points), indicating particularly acute male underrepresentation in childcare despite contemporary rhetoric around "involved fatherhood."

These patterns persist across socioeconomic strata and life stages. Education and urban residence—typically associated with egalitarian gender attitudes—do not narrow care work gaps. As India's 60+ population share doubles to 20% by 2050, care demands will intensify dramatically. Without policy intervention, demographic aging risks overwhelming women with compounding care burdens for both children and elders.

7 References

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